

Telehealth Requirements – IT and Compliance

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Today's Faculty: SONAL PATEL



SONAL PATEL

Sonal has over 15 years of experience understanding the art of business medicine. She is a nationally recognized thought-leader, speaker, author, creator, and consultant. She is the founder of SP Collaborative, LLC.

Sonal is an active member with the AAPC®, the American Medical Billing Association (AMBA®), Health Care Compliance Association (HCCA®), NAMAS®, and Practice Management Institute (PMI®).

Sonal is currently on the BC Advantage Editorial Board. Sonal is also a published author in numerous industry publications, including AAPC's *Healthcare Business Monthly* and *Knowledge Center Blog*, *Lexology*, *JD Supra*, as well as two chapters in HCCA's 2021 edition of the *Complete Healthcare Compliance Manual*.

She serves as the creator and host of her weekly Paint The Medical Picture Podcast series which is currently in its fifth year.



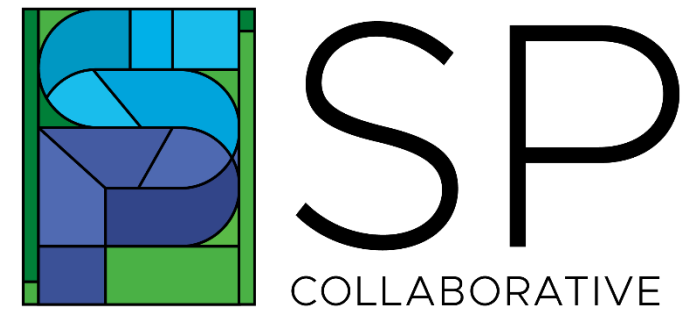
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Learning Objectives



Learning Objectives

IT and Compliance

HIPAA

Telehealth Security

Remote Patient Monitoring Set Up

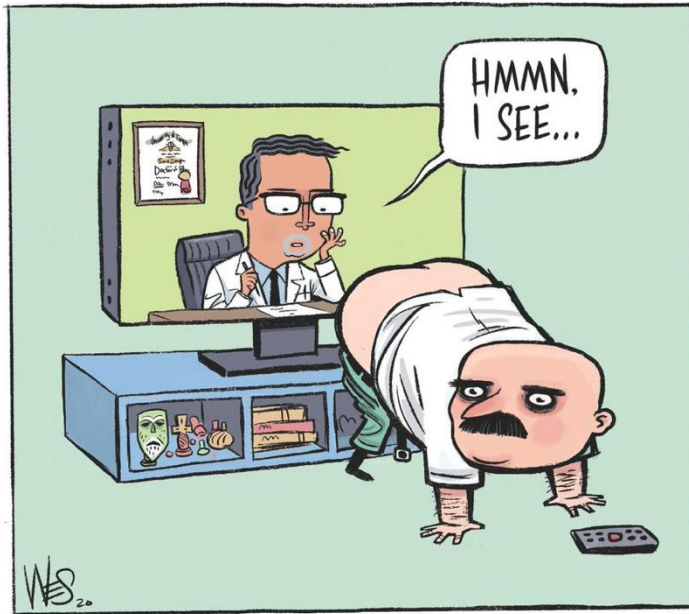
Key Elements of an Effective Compliance Program

Final Thoughts

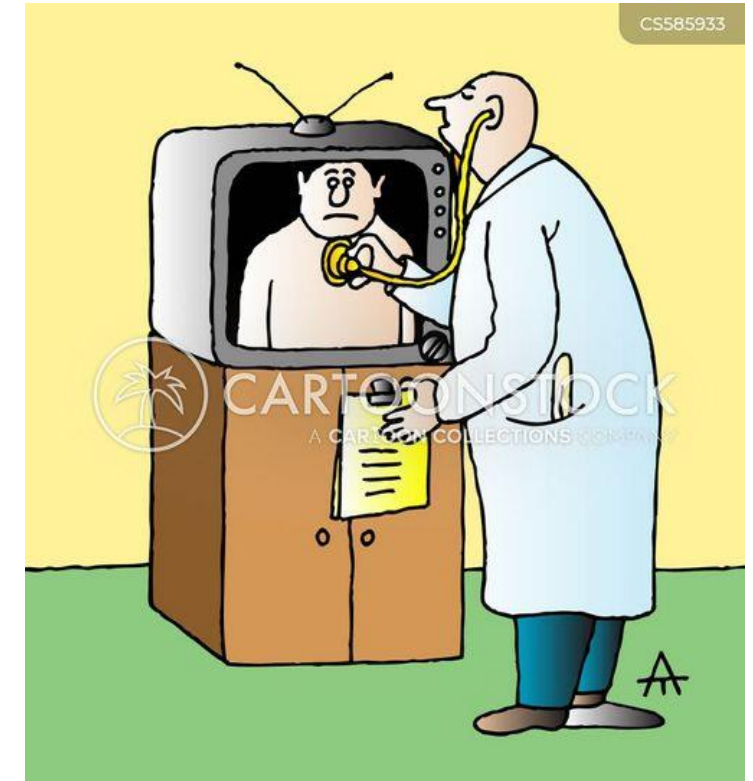
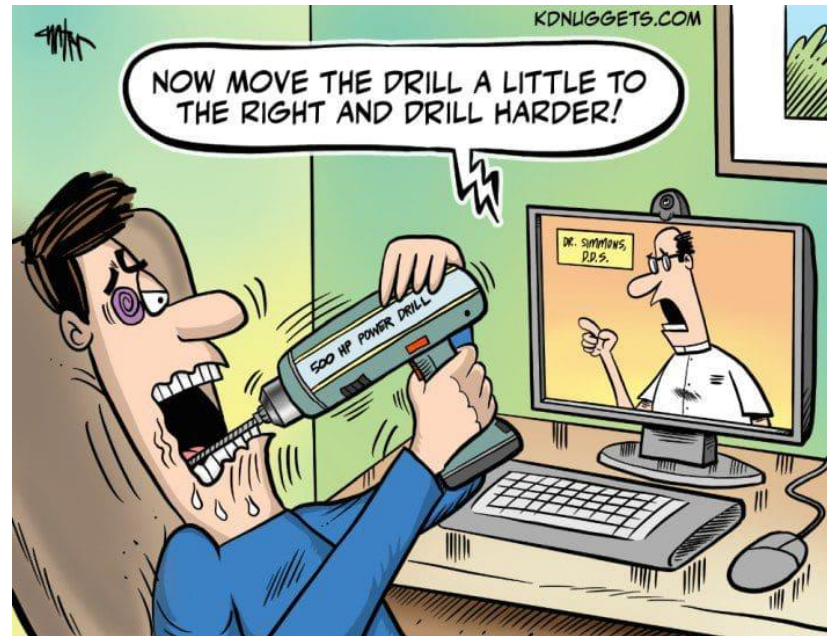


IT and Compliance

Adventures of Telehealth – When is it Appropriate?



THE DOCTOR IS IN



IT in Telehealth





IT in Telehealth

1876 – Invention of Telephone

- “Telemedicine has been a factor in the provision of healthcare since the invention of the telephone. As video and digital technologies evolved throughout the twentieth century, healthcare providers adopted the technologies to improve the quality of remote care – not only by replacing physical visits with virtual ones, but also by facilitating faster collaboration between healthcare units.

2010 - ACA

- The adoption of physician-to-patient telemedicine accelerated following the passage of the Affordable Care Act and the subsequent Hospital Readmissions Reduction Program introduced by the Centers for Medicare and Medicaid Services (CMS). Healthcare providers found that, by monitoring patients’ health remotely, hospital readmissions dropped significantly – generating massive savings.” – HIPAA Journal

IT in Telehealth

Year	Policy	Flexibilities	Status
2020	Covid19 PHE Begins	Originating site waived (home allowed) – Audio-only allowed – More provider types	PHE only
2021-2022	CMS & Congress Extend Flexibilities	Continued coverage – Payment parity for telehealth	PHE only
2023	CAA 2023	Extended telehealth flexibilities thru 12/31/2024 and again thru 12/31/2025 – delayed in-person requirement for telehealth mental	Statutory extension
3/31/2025	1 st Congressional Extension Deadline	Continued waivers for originating site and geographic locations – audio-only still allowed	Extended
9/30/2025	Latest Extension Deadline	Flexibilities Expired 10/1/2025	Expired

IT in Telehealth

Year	Policy	Flexibilities	Status
10/1/2025 -11/12/2025	Government Shutdown (43 days)	Expired (some exceptions)	Expired
10/31/2025	FY 2026 MPFS Final Rule	Extended telehealth flexibilities thru 1/30/2026	Extended



HIPAA



HIPAA Requirements for Telehealth

- **Platform and vendor compliance:** Healthcare providers must use telehealth platforms that are compliant with HIPAA rules and enter into business associate agreements with the vendors.
- **Risk analysis:** A thorough risk analysis is necessary to identify and mitigate potential vulnerabilities to patient health information, which may require different safeguards for virtual services than for in-person care.
- **Privacy Rule:** Providers must adhere to the HIPAA Privacy Rule, which includes standards for patient consent, uses and disclosures of health information, and the patient's right to access their records.



HIPAA Requirements for Telehealth

- **Security Rule:** The Security Rule requires providers to implement administrative, physical, and technical safeguards to protect electronic protected health information (ePHI). This includes using secure communication channels, encrypting data, and ensuring only authorized users can access ePHI.
- **Patient consent:** Providers must obtain appropriate consent from patients before sharing their information, especially for uses beyond treatment, payment, or healthcare operations.
- **Minimum necessary standard:** The rule mandates that only the minimum necessary amount of PHI is used or disclosed for a specific purpose, which is particularly important for digital data transmission.



HIPAA Breaches

- As of October 22, 2025, OCR has added 26 data breaches affecting 500 or more individuals to its data breach portal – the lowest monthly total since December 2018.

The Biggest Healthcare Data Breaches Announced in September

Currently, 42% of the month's breaches (11 incidents) involved the exposure or impermissible disclosure of the protected health information of 10,000 or more individuals. All but one of the 11 data breaches were hacking incidents involving unauthorized access to protected health information stored on network servers, with one incident involving a compromised email account. Goshen Medical Center was the worst-affected covered entity, with more than 456,000 patients affected by its hacking incident. One provider that stands out is Sturgis Hospital, which was investigating a cyberattack that occurred in December 2024, when another intrusion was experienced in June 2025.



Telehealth Security



Telehealth Security

- **For patients:**
- Use a private location: Ensure your visit takes place in a private, enclosed area where you can control who sees or hears you.
- Use a personal device: Avoid using a device or network that is tied to your workplace or a public setting.
- Secure your device: Protect your devices with strong passwords and up-to-date antivirus software. Do not use public Wi-Fi for telehealth appointments.
- Use secure connections: Look for a lock icon in the website address bar before entering personal information. Make sure your home Wi-Fi is password-protected.
- Be wary of phishing: Do not provide personal information via phone or text from requests not associated with your practice.
- Delete unnecessary information: Securely delete health information from devices once you no longer need it.



Telehealth Security

- **For providers:**
- **Verify patient identity:** Confirm the identity of patients before the visit begins.
- **Use a secure platform:** Utilize a HIPAA-compliant telehealth platform that uses encrypted communication.
- **Secure your network:** Conduct sessions in a secure space with private Wi-Fi. Limit access to session personnel.
- **Protect devices and data:** Ensure your devices are password-protected and have the latest security software. Avoid using conventional email or text for sensitive information.
- **Educate patients:** Advise patients on how to maintain their own privacy during telehealth visits.



RPM Set Up



Remote Patient Monitoring

Remote Patient Monitoring

Remote patient monitoring (RPM) allows a patient to collect their own health data (for example, blood pressure) using a connected medical device that automatically transmits the data to their provider. The provider then uses this data to treat or manage the patient's condition. RPM includes both remote physiological monitoring and remote therapeutic monitoring.

- **Remote physiological monitoring** involves using non-face-to-face technology to monitor and analyze a patient's physiological metrics. Examples of physiological metrics include:
 - Oxygen saturation
 - Blood pressure
 - Blood sugar or blood oxygen levels
 - Weight loss or gain
- **Remote therapeutic monitoring (RTM)** captures non-physiological data, often self-reported, related to a therapeutic treatment. This includes data on a patient's musculoskeletal or respiratory system. RTM can also monitor treatment adherence and treatment response. A connected medical device transmits the patient's information.



Remote Patient Monitoring

RPM Requirements

- Remote physiologic monitoring, but not RTM, requires an established patient relationship
- Only physicians and non-physician practitioners eligible to provide evaluation and management services can bill RPM services
- Remote physiologic monitoring:
 - You must monitor an acute or chronic condition
 - You must collect data for at least 16 days out of 30 days (doesn't apply to treatment management codes 99457, 99458, 98980, and 98981)
- Only 1 practitioner can bill for RPM per patient in a 30-day period
- You can't bill remote physiologic monitoring and RTM together
- Monitoring must be medically reasonable and necessary
- You may bill remote physiologic monitoring and RTM, but not both, concurrently with the following care management services for the same patient if you don't count time and effort twice: chronic care management, transitional care management, behavioral health integration, principal care management, and chronic pain management
- Practitioners who aren't receiving the global periods of surgery service payment can bill for RPM services
- We require patient consent at the time you provide RPM services
- You must electronically collect physiologic data and automatically upload it to a secure location where the data can be available for analysis and interpretation by the billing practitioner
- The device used to collect and transmit the data must meet the definition of a medical device [defined by FDA](#)
- Auxiliary personnel can provide RPM services under the general supervision of the billing practitioner



Remote Patient Monitoring

RPM Components

RPM consists of 3 main components, each building off the step before it.

1. Patient education and device setup: How to use the device; how to accurately collect data
2. Device supply: Device examples; connecting the device so you can read results; how often patients should use devices
3. Treatment management: Reviewing patient data to improve patient health outcomes

Remote Patient Monitoring

CPT/HCPCS Code	Description	Time	Audio-only coverage
99091	Monthly review of data	30 minutes	N/A
99453	Initial setup and monitoring	N/A	N/A
99454	Monthly review of RPM data	16 or more days over a 30-day period	N/A
99457	Patient-provider communication related to RPM data	20 minutes	Yes
99458	Patient-provider communication related to RPM data	Additional 20 minutes	Yes
98975	RTM device setup and patient education	N/A	N/A
98976	RTM monitoring, respiratory	16 or more days over a 30-day period	N/A
98977	RTM monitoring, musculoskeletal	16 or more days over a 30-day period	N/A
98980	Patient-provider communication related to therapeutic device	20 minutes	Yes
98981	Additional time required for 98975–98978 or 90980	Additional 20 minutes	Yes



Remote Patient Monitoring

- Compliance Risks for:
 1. Lack of prior relationship
 2. Insufficient documentation – include consent, communication mode, locations, time, medical necessity
 3. Code Selection
 4. Eligibility
 5. Billing Patterns



Program Integrity Risk

Compliance Risks Continue if Compliance Programs Not Updated

- 5 Fraud and Abuse Laws that could be triggered:
- False Claims Act (FCA) - 31 U.S.C. §§ 3729-3733
- Anti-Kickback Statute (AKS) - 42 U.S.C. § 1320a-7b(b)
- Stark Law - 42 U.S.C. § 1395nn
- Exclusion Statute - 42 U.S.C. § 1320a-7
- Civil Monetary Penalties - 42 U.S.C. § 1320a-7a
- Commercial payors also have watch dogs – the SIU



OIG Audit on Billing for RPM

Billing for Remote Patient Monitoring in Medicare

Issued on 08/25/2025 | Posted on 08/28/2025 | Report number: OEI-02-23-00261



OIG Audit on Billing for RPM

Why OIG Did This Review

- Remote patient monitoring is a technology-based health care service designed to allow for better management of health conditions, especially chronic conditions.
- Remote patient monitoring allows a patient to collect their own health data (e.g., their blood pressure or weight) using a connected medical device that automatically transmits these data to their provider. The provider then uses these data to treat or manage the patient's condition.
- OIG previously found that the use of these services in Medicare has the potential to greatly expand in the future and that additional oversight of remote patient monitoring is needed.



OIG Audit on Billing for RPM

What **OIG** Found

- The use of remote patient monitoring continued to grow in 2024, with Medicare payments exceeding \$500 million.
- Monitoring billing can help safeguard the Medicare program and prevent fraud, waste, and abuse.
- We developed several measures to monitor billing for remote patient monitoring including:
 - billing for a high proportion of enrollees who have no prior history with the medical practice; and
 - billing for multiple monitoring devices a month for an enrollee.
- These measures can identify medical practices with billing for remote patient monitoring that warrant further scrutiny.
- Analyzing billing can help CMS, Medicare Advantage Organizations, and other entities ensure that enrollees receive the benefit of remote patient monitoring while, at the same time, minimizing program integrity risks.

OIG Audit on Billing for RPM

Nearly 1 million Medicare enrollees received remote patient monitoring in 2024.





Holter Monitor as Remote Patient Monitoring

- Holter monitors are a type of remote cardiac monitor (RCM) that integrates with telehealth by sending heart activity data to healthcare providers for remote analysis.
- This allows for continuous monitoring outside of a clinic, enabling early detection of irregularities and reducing the need for in-person visits.
- Telehealth platforms facilitate this by providing a connection between the monitor, a mobile app, and a cloud-based platform where doctors can view data and consult with patients, even if the patient is at home.



Holter Monitor as Remote Patient Monitoring

- The CPT codes for Holter monitoring are 93224-93227 for up to 48 hours of continuous recording. 93224 is the global code for the full service, while 93225 is for the recording, 93226 for the analysis, and 93227 for the physician's interpretation. For monitoring longer than 48 hours but up to seven days, different codes like 93241 (global) apply.



Holter Monitor as Remote Patient Monitoring

How they work together

- **Data Collection:** A Holter monitor is a wearable device that records a patient's heart activity for an extended period.
- **Data Transmission:** The device transmits the recorded data to a connected mobile app, which then sends it to a cloud platform.
- **Remote Analysis:** Healthcare providers can access this data on the cloud platform to remotely monitor the patient's heart function.
- **Patient Interaction:** The telehealth system allows for virtual communication, such as video calls, to discuss findings or guide patients on using the equipment.



Holter Monitor as Remote Patient Monitoring

Benefits of combining Holter monitors and telehealth

- Continuous Monitoring: Enables ongoing tracking of a patient's heart activity in their home environment.
- Early Intervention: Allows for quicker detection of problems and earlier intervention by clinicians, potentially preventing more serious events.
- Improved Patient Outcomes: Reduces the need for hospital visits and readmissions.
- Enhanced Patient Experience: Provides convenience for the patient, who can be monitored from home.
- Greater Efficiency: Saves time for clinics and can lead to faster care for patients.

Holter Monitor as Remote Patient Monitoring

- Compliance Met
- Established patient relationship
- Treatment set up





Keys Element of an Effective Compliance Program



Key Elements of an Effective Compliance Program

Risk
Assessment,
Auditing &
Monitoring

Training &
Education



Key Elements to an Effective Compliance Program

Training & Education

The Compliance Officer and Compliance Committee develop and coordinate a multifaceted education and training program specific to the practice
All board members, officers, employees, contractors, and medical staff (if applicable) of the entity should receive training at least annually on the entity's compliance program and potential compliance risks.

Training & Education

Targeted training sessions may include: billing, coding, documentation, medical necessity,
The education and training program also should include a requirement that licensed personnel must complete all education and training mandated by the licensing board that governs their license.

Key Elements to an Effective Compliance Program

Auditing

- Auditing based on identified risks
- Auditing based on overpayment
- Auditing for quality assurance – procedure and diagnosis coding and/or patient safety
 - Proactive auditing or self-auditing





Final Thoughts



Final Thoughts

- **Policy Shift** - Huge shift back to telehealth BEFORE the pandemic for Medicare population and then back to PHE times until 1/30/2026
- **Government Shutdown Trickle Down Effect** - Practices were uncertain of next steps
- **States Fly Their Own Flags** – So much variance
- **Compliance** – What's That? Embrace it.
- **Due Diligence** – Continued responsibility
- **C-Suite** – Leadership decisions and strategic moves



Resources



Resources

- <https://www.hipaajournal.com/hipaa-guidelines-on-telemedicine/>
- <https://www.hhs.gov/hipaa/for-professionals/special-topics/telehealth/index.html>
- <https://www.hipaajournal.com/september-2025-healthcare-data-breach-report/>
- <https://www.hhs.gov/hipaa/for-professionals/privacy/guidance/telehealth-privacy-security/index.html>
- <https://www.cms.gov/files/document/mln901705-telehealth-remote-patient-monitoring.pdf>
- <https://oig.hhs.gov/compliance/physician-education/fraud-abuse-laws/>
- <https://oig.hhs.gov/reports/all/2025/billing-for-remote-patient-monitoring/>
- CPT® Coding Manual 2025, Professional Edition

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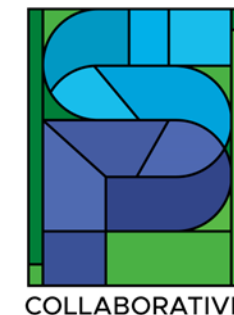
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◆ CEO & Principal Strategist ◆ Healthcare Co...
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#ThursdayThoughts 💬 💬

My thoughts this month center around what bravery means - even when facing difficulties.

👉 Believe in yourself even when everyone has left.

"Courage starts with showing up and letting ourselves be seen." — Brené Brown

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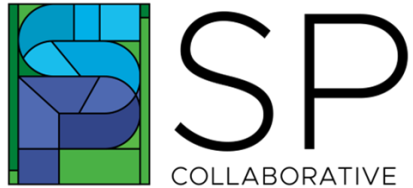
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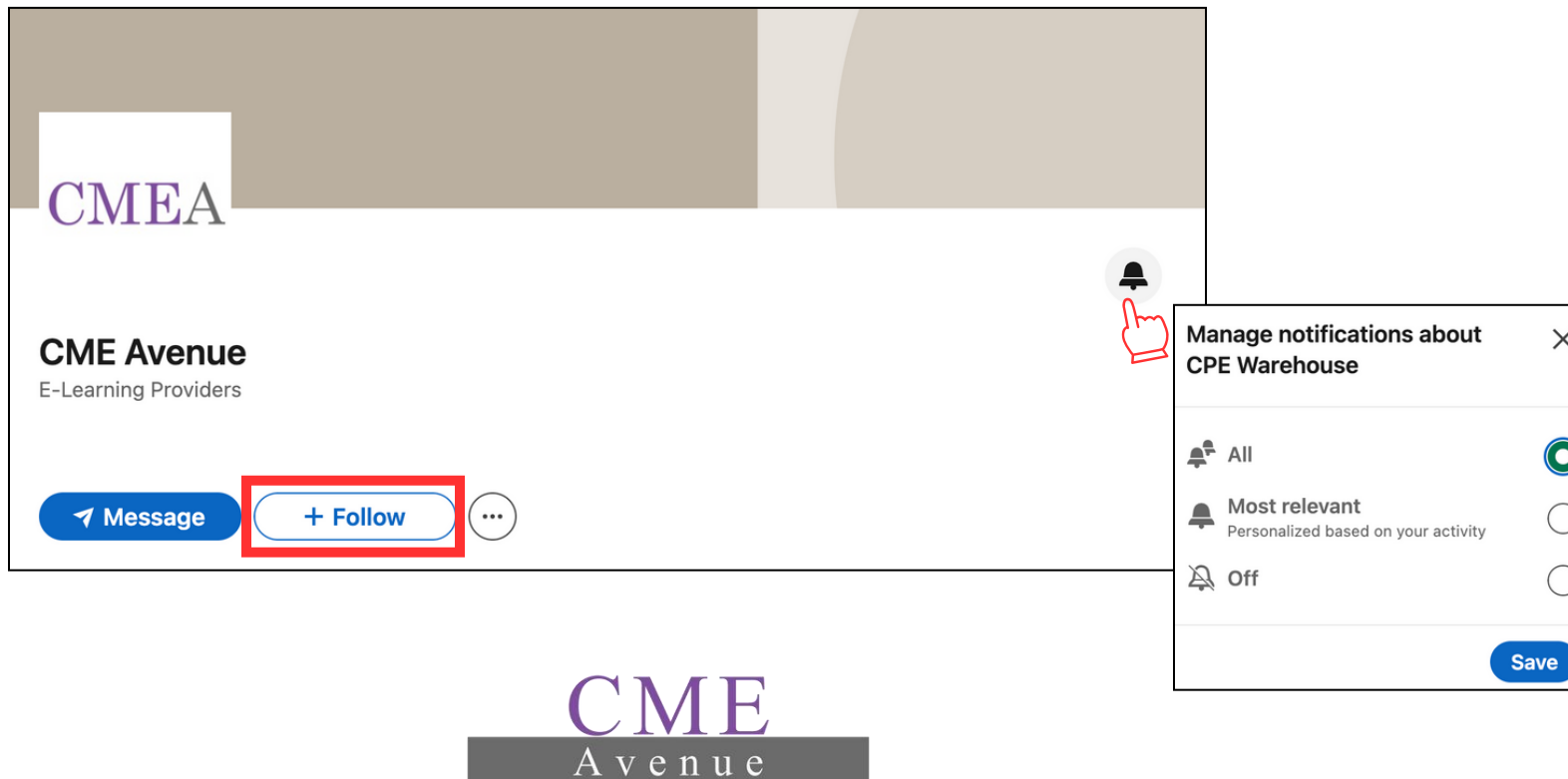
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